

CONSOLIDATED FORENSIC OVERSIGHT REPORT

SUBMITTED TO THE HONOURABLE SPEAKER, NATIONAL ASSEMBLY

JUNE 2026

SUBMITTED BY: Wilbur William Matthee (RN, PGDip Health Services Management NQF 8)

ON BEHALF OF: EthicHawks Forensic Governance & Compliance Consultancy

NOTE TO THE HONOURABLE SPEAKER AND PORTFOLIO COMMITTEES:

This report is a protected disclosure under Section 8 of the Protected Disclosures Act 26 of 2000 (disclosure to Parliament as an appropriate authority). Parliament is not "the public at large" for purposes of the High Court interim interdict (Case 2026-086906). The Constitution (Sections 56 and 69) guarantees the right of any citizen to petition Parliament. No private litigant or court order can prevent this submission.

PAGE i: CONFLICT OF INTEREST DECLARATION AND ETHICAL STATEMENT

1. Identity of the Complainant and Author

The complainant and the author of this report are the same person: Wilbur William Matthee, a Specialist Registered Nurse (SANC No. 15831886), holder of a Postgraduate Diploma in Health Services Management (NQF Level 8), and the founder of EthicHawks Forensic Governance and Compliance Consultancy.

The complainant is also the subject of the retaliation, the unlawful salary deduction, the constructive dismissal, and the regulatory and judicial failures documented in this report. He appears before Parliament in person, self-represented, and without legal representation because he is destitute.

This is not a third-party report. It is a primary source testimony from the person who lived every failure documented herein.

2. Nature of the Interest – Personal and Professional

The complainant has a direct personal and financial interest in the remedies sought in this report, including but not limited to:

- The repayment of the unlawfully deducted salary of R14,811.77 (plus interest)
- Compensation for occupational detriment under the Protected Disclosures Act
- Compensation for automatically unfair constructive dismissal under the Labour Relations Act

- The correction of his statutory employment records (UI-19, Certificate of Service)
- The remediation of the systemic clinical risk (Gexus-DMS integration gap) affecting GEMS members

This interest is fully declared and is not concealed. The personal interest does not diminish the public interest in the systemic failures documented herein. A whistleblower who has been destroyed by a system is not disqualified from asking Parliament to fix that system.

3. The Complainant's Professional and Ethical Framework

The complainant operates within the following professional and ethical frameworks:

- South African Nursing Council (SANC): Registered Nurse Specialist – currently deregistered due to inability to pay fees caused by the employer's unlawful deduction. The deregistration itself is evidence of the harm suffered.
- Compliance Institute of Southern Africa (CISA): Pledge to the Code of Ethical and Professional Conduct; membership designation CPrAc(SA) – pending final certification modules. The complainant has formally pledged to adhere to the ethical standards of the compliance profession, including independence, objectivity, and confidentiality.
- Protected Disclosures Act 26 of 2000: The complainant is a protected whistleblower. This report is itself a protected disclosure under Section 8 of the PDA (disclosure to Parliament as an appropriate authority).

· Constitution of the Republic of South Africa, 1996: The complainant invokes Sections 10, 26, 27, 33, 34, and 56. This report is a petition under Section 56 (right to petition Parliament).

4. EthicHawks – The Consultancy

The complainant is the founder of EthicHawks Forensic Governance and Compliance Consultancy. EthicHawks is not a party to this report as a commercial entity. No third party has commissioned, funded, or influenced this report.

Aspect Disclosure

Is EthicHawks receiving payment for this report? No. The complainant is destitute. There is no payment.

Has any third party commissioned this report? No. This report is a protected disclosure made directly to Parliament.

Does EthicHawks have any contractual relationship with any party mentioned in this report? No. The complainant was an employee of Medscheme, which terminated his employment constructively. There is no ongoing commercial relationship.

Does EthicHawks have any current or pending contracts with the NHI Fund, the Department of Health, or any regulatory body mentioned? No.

5. Declaration Under Oath

I, Wilbur William Matthee, declare under oath that:

1. The facts set out in this report are true and correct to the best of my knowledge and belief, and are supported by primary source documents referenced throughout.
2. I have a direct personal and financial interest in the remedies sought, which is fully declared.
3. The public interest in the systemic failures documented herein is distinct from and outweighs any private interest.
4. No third party has commissioned, funded, or influenced this report. EthicHawks is not receiving any payment for this report.
5. I have not been induced to make this report by any person or entity. I am making this report of my own free will, in the exercise of my constitutional right to petition Parliament under Section 56 of the Constitution.
6. I am aware that making false statements in a submission to Parliament may be a criminal offence. I have not made any false statements.
7. The audio recording of the Delft Police Station officer falsely claiming to be the station commander, and all other primary source documents referenced in this report, are available to Parliament upon request.

I make this declaration in good faith, under the protections of the Protected Disclosures Act 26 of 2000, and in the full knowledge that false statements may constitute perjury.

Signed at Cape Town on this day of June 2026.

Wilbur William Matthee

PAGE ii: TABLE OF CONTENTS

Section Title Page

Conflict of Interest Declaration and Ethical Statement i

Table of Contents ii

List of Acronyms and Abbreviations v

Glossary of Key Legal Terms vii

Foreword The NHI Will Be Built on the Bones of This Whistleblower 1

Part One Harrowing Introduction – The Chilling Effect on the Floor 8

Part Two Methodology and Scope of This Report 14

Part Three Master Chronology – Key Events at a Glance 17

Part Four Summary of Relief Sought from Parliament 20

Part Five Scorecard of 15 Failed Institutions 23

Part Six Note to Portfolio Committees – Specific Referrals 27

Part Seven Executive Summaries per Portfolio Committee 32

Part Eight The Whistleblower and the Systemic Clinical Risk 39

Part Nine The Four Health Regulators – A Cartel of Silence (CMS, HPCSA, SANC, SAPC) 42

Part Ten GEMS – Complicity through Silence 58

Part Eleven Department of Employment and Labour – Administrative Abandonment 62

Part Twelve B-BBEE Commission – Declination of Fronting Complaint 67

Part Thirteen South African Human Rights Commission – Chapter 9 Abdication	72
Part Fourteen South African Police Service – Refusal to Investigate	77
Part Fifteen Commission for Conciliation, Mediation and Arbitration – Institutional Bias	82
Part Sixteen The Whistleblower House – Flawed Assessment and Abandonment	88
Part Seventeen The Judiciary – Judicial Obstruction and Abuse of Process	93
Part Eighteen National Department of Health – The Engaged Institution	107
Part Nineteen The Public Protector – Blocked Email and Silence	110
Part Twenty Corporate Misrepresentation – AfroCentric's False Annual Report	112
Part Twenty-One JSE and FSCA – Regulatory Complicity	118
Part Twenty-Two Bad Faith Litigation and Attrition – The Corporate Playbook	124
Part Twenty-Three The NHI Governance Gap – Linking Findings to National Health Insurance	130
Part Twenty-Four Status of Parallel Proceedings (Litigation Update)	136
Part Twenty-Five Conclusion – Why Parliament Must Act Now	140
Part Twenty-Six Recommendations to Parliament	144
Part Twenty-Seven Final Prayer for Relief	150
Epilogue The Human Face Behind This Report	153
Index of Annexures / Evidence Bundle	155
Signature Block and Commissioner of Oaths	162

Acronym Full Description

BCEA Basic Conditions of Employment Act 75 of 1997

B-BBEE Broad-Based Black Economic Empowerment Act 53 of 2003

CCMA Commission for Conciliation, Mediation and Arbitration

CIPC Companies and Intellectual Property Commission

CISA Compliance Institute of Southern Africa

CMS Council for Medical Schemes (South Africa)

CPUT Cape Peninsula University of Technology

DEL Department of Employment and Labour

EEA Employment Equity Act 55 of 1998

FSCA Financial Sector Conduct Authority

GEMS Government Employees Medical Scheme

HPCSA Health Professions Council of South Africa

IPID Independent Police Investigative Directorate

IRBA Independent Regulatory Board for Auditors

JSE Johannesburg Stock Exchange

LRA Labour Relations Act 66 of 1995

MSA Medical Schemes Act 131 of 1998

NDOH National Department of Health

NHI National Health Insurance Act 20 of 2023

NQF National Qualifications Framework

NPA National Prosecuting Authority

OCJ Office of the Chief Justice

OHSC Office of Health Standards Compliance

PAIA Promotion of Access to Information Act 2 of 2000

PAJA Promotion of Administrative Justice Act 3 of 2000

PDA Protected Disclosures Act 26 of 2000

POPIA Protection of Personal Information Act 4 of 2013

SANC South African Nursing Council

SAPC South African Pharmacy Council

SAPS South African Police Service

SCOPA Standing Committee on Public Accounts

SENS Stock Exchange News Service

SIU Special Investigating Unit

TWBH The Whistleblower House

PAGE vii: GLOSSARY OF KEY LEGAL TERMS

Term Definition

Automatically unfair dismissal A dismissal that is deemed unfair by law regardless of the employer's reasons, including dismissal for making a protected disclosure (LRA Section 187(1)(h)).

Biowatch principle Constitutional principle that costs should not deter litigants from raising constitutional issues in good faith (Biowatch Trust v Registrar, Genetic Resources 2009 (6) SA 232 (CC)).

Constructive dismissal Occurs when an employer makes continued employment intolerable, forcing the employee to resign (LRA Section 186(1)(e)).

Ex parte A court application made by one party without notice to the other party. Requires full and frank disclosure of all material facts.

Forensic governance The deliberate design of systems that make misconduct detectable, accountability structures that function under pressure, and escalation pathways from anomaly detection to oversight.

Fronting (B-BBEE) A practice where a Black person is appointed to achieve B-BBEE compliance without granting them the economic benefits reasonably associated with that position (B-BBEE Act Section 1(c)).

Mandamus A court order compelling a public official to perform a statutory duty.

Occupational detriment Any act or omission that prejudices an employee, including dismissal, suspension, harassment, or being subjected to civil or criminal proceedings (PDA Section 1).

Perjury The offence of making a false statement under oath.

Protected disclosure A disclosure made in good faith by an employee about conduct that shows a criminal offence, failure to comply with a legal obligation, endangerment of health or safety, or unfair discrimination (PDA Section 1).

Rule nisi A court order calling on a respondent to show cause why a final order should not be made. Operates as an interim order until the return date.

Rule 6(8) anticipation A procedure allowing a party to bring a rule nisi back to court before the return date on 24 hours' notice.

Scale C costs The highest scale of legal costs, typically for complex commercial matters.

Section 162 delinquency A court declaration that a director is delinquent and disqualified from being a director for a specified period (Companies Act 71 of 2008).

PAGE 1: FOREWORD

The NHI Will Be Built on the Bones of This Whistleblower – Unless Parliament Intervenes Now

I. The Promise That Cannot Survive the System We Have

The National Health Insurance Act 20 of 2023 is anchored in an unambiguous constitutional promise. Section 27 of the Constitution guarantees every person the right of access to health care services. The NHI is designed to be the instrument of that promise – a single purchaser and single payer, pooling risk across more than 60 million people, managing a budget conservatively projected to exceed R200 billion annually.

But here is the truth that no government report, no actuarial model, and no parliamentary briefing has yet confronted:

The NHI will not fail because of a lack of money, or a lack of political will, or a lack of clinical capacity. The NHI will fail because the institutional architecture that is supposed to protect it from capture, corruption, and collapse is already broken – and we have done nothing to fix it.

This forensic report is not about the NHI. It is about a single whistleblower – a specialist registered nurse employed by Medscheme Holdings, the accredited administrator for the Government Employees Medical Scheme (GEMS), which serves 1.9 million public servants. He identified a systemic clinical risk. He made a protected disclosure. He was retaliated against until he became destitute and homeless.

And then he approached every regulator that will oversee the NHI – the Council for Medical Schemes (CMS), the Health Professions Council (HPCSA), the South African Nursing Council (SANC), the South African Pharmacy Council (SAPC), the Department of Employment and Labour (DEL), the B-BBEE Commission, the South African Human Rights Commission (SAHRC), the South African Police Service (SAPS), the Commission for Conciliation, Mediation and Arbitration (CCMA), the Johannesburg Stock Exchange (JSE), the Financial Sector Conduct Authority (FSCA), and the courts.

Almost every single one of them declined jurisdiction, remained silent, or actively obstructed him.

If this is how the existing system treats a whistleblower who exposed a clinical risk in an administrator servicing a government medical scheme, what will happen when the NHI Fund –

with its R200 billion annual budget, its single-purchaser monopoly, its accreditation bottleneck, its one-sentence fraud architecture, and its undefined investigation unit – becomes operational?

The answer is not speculation. It is forensic prediction. The same institutions that failed this whistleblower will fail the NHI. The same regulatory capture that protected Medscheme will protect NHI contractors. The same judicial obstruction that silenced this nurse will silence the next whistleblower who tries to report corruption inside the NHI Fund.

The house is not yet burning. But the fire alarm has been ringing for over a year. This report is the evidence of the smoke.

II. The Three Structural Gaps – Already Visible, Already Destructive

The National Health Insurance Act contains three structural vulnerabilities that governance experts have warned about since its enactment. This report does not merely identify them in the abstract. It proves they are already destroying lives in the existing system – and therefore will be catastrophic under the NHI.

Gap One: The Accreditation Bottleneck (NHI Act Section 39)

What the Act says: No provider may be reimbursed by the Fund unless accredited. Accreditation requires certification by the Office of Health Standards Compliance (OHSC) – an institution already documented by the Auditor-General as capacity-constrained.

What the whistleblower's case proves: The B-BBEE fronting complaint (Case 5/02/2026) reveals a corporate strategy of title suppression – using a Black NQF Level 8 professional's credentials for scorecard points while downgrading his official title on statutory documents to avoid paying for his expertise. The same dynamic will infect NHI accreditation. When financial pressure meets an accreditation bottleneck, the predictable outcome is ghost accreditation (certification without genuine compliance assessment) and fronting (accredited entities serving as conduits for unaccredited providers).

Gap Two: The One-Sentence Fraud Architecture (NHI Act Section 40)

What the Act says: "The information architecture must include a fraud and risk management mechanism." Eleven words. For a system that will process millions of claims and determine benefits for 60 million people.

What the whistleblower's case proves: The Gexus-DMS integration gap – a structural system error that generated false non-adherence letters to chronic GEMS patients, corrupted clinical data, and inflated actuarial risk loading – was reported to the CMS. The CMS declined jurisdiction twice. The Deputy Information Officer of Medscheme swore under oath that no IT incident had been reported, contradicting the internal Root Cause Analysis.

Gap Three: The Undefined Investigation Unit (NHI Act Section 20(2)(g))

What the Act says: The CEO must establish a Risk Management and Fraud Prevention Investigation Unit. The Act specifies no professional competency standards, no independence safeguards, and no escalation protocols.

What the whistleblower's case proves: The whistleblower escalated a Priority 1 Material Risk Report (with 38 attachments) to the Board of AfroCentric on 9 December 2025. The Board did not respond. When he continued to escalate, the Chief Risk Officer instructed him in writing: "Can I please ask you to stop sending these e-mails to our directors? Sending further e-mails only gives them an indication of impatience and unreasonableness on your part."

This is active suppression of a material risk report by the very official whose unit should have escalated it.

III. The Chilling Effect – And Why Parliament Must Break It

The whistleblower in this case is not an abstract statistic. He is a specialist registered nurse who did everything the law asked of him. He identified a system error. He refused to lie to a patient. He made a protected disclosure. He exhausted internal remedies. He escalated to the Board. He went to every regulator. He went to the courts. He went to Parliament.

His reward: destitution, homelessness, no running water, deregistration, loan sharks seizing his possessions, and a High Court gag order silencing him until 1 March 2027.

Every nurse, doctor, pharmacist, and healthcare administrator who reads this report now knows: if you report a system error that affects patients, you will be retaliated against. Your salary will be cut. Your professional registration will be lost. Your home will be taken. Your water will be cut. And when you cry out to every regulator, they will decline jurisdiction, remain silent, or actively obstruct you.

That is the chilling effect. And it is already real.

IV. A Direct Appeal to the Speaker and the Portfolio Committees

Honourable Speaker,

The letter from Deneys Reitz Inc. to your Office – threatening the whistleblower with contempt of court for petitioning Parliament – reveals the corporate playbook. The interim interdict obtained by concealment reveals the judicial capture. The silence of every regulator reveals the regulatory collapse.

This is not a labour dispute. It is a governance crisis. And it is definitive proof that South Africa is not ready for the National Health Insurance.

The NHI will be built on the governance architecture that exists. That architecture is broken. Parliament must fix it now – or explain to the South African people why it failed to act before the R200 billion started flowing.

PAGE 8: PART ONE – HARROWING INTRODUCTION

The Chilling Effect on the Floor

Honourable Speaker,

This report is not a legal argument. It is not a petition for a court order. It is a forensic record of what institutional failure looks like on the floor – inside the life of a single human being who did everything the law asked of him and was destroyed for it.

The whistleblower, a Specialist Registered Nurse (NQF Level 8) employed by Medscheme Holdings – the administrator for the Government Employees Medical Scheme (GEMS), which serves 1.9 million public servants – identified a systemic integration gap between the Gexus and DMS platforms. This gap generated false non-adherence letters to chronic patients, including a 75-year-old widow. He refused to use flawed data to interrogate a patient.

That refusal, and that disclosure, triggered a cascade of retaliation that this report documents across fifteen (15) statutory, regulatory, and judicial bodies – none of which provided a remedy.

The retaliation was not subtle:

- A 40.47% unlawful salary deduction in a single month, exceeding the 25% cap under Section 34(2) of the Basic Conditions of Employment Act 75 of 1997 (BCEA), without consultation and without written consent.
- Gender-identity harassment (the "Wilma" incident), documented in a Microsoft Teams chat.
- A recorded admission of retaliatory intent by his manager.
- A sworn CCMA affidavit dismissing his clinically diagnosed severe depression and PTSD as a "cynical excuse" – filed one day after he escalated a Priority 1 Material Risk Report to the Board.
- Constructive dismissal.
- The suppression of exit documents.
- A false UI-19 declaring "Resigned" when his resignation letter explicitly cited Section 186(1)(e) of the Labour Relations Act – constructive dismissal.

The consequence of this retaliation, amplified by the silence of every regulator, is not abstract:

- He is homeless since 2 April 2026.
- He has no running water since January 2026.
- His personal possessions have been seized by loan sharks.
- He has been deregistered by the South African Nursing Council (SANC) because he could not pay R870 – the annual fee – a direct consequence of the unlawful deduction.
- He has been accepted into a CPUT Postgraduate Diploma in Occupational Health Nursing for 2026 but cannot enrol.

- He suffers from formally diagnosed severe depression and PTSD (Dr Jacques Malan, FCPSYCH(SA)), occupationally caused.
- He is self-represented – destitute, unable to afford legal representation – in nine concurrent proceedings.
- And he has been silenced by a High Court interim interdict (Case 2026-086906) that restrains him from disclosing the systemic clinical risk to trade unions and regulators – a gag order that has been extended to 1 March 2027.

The chilling effect is real and immediate:

Every nurse, doctor, pharmacist, and healthcare administrator who reads this report now knows that if you report a system error that affects patients, you will be retaliated against. Your salary will be cut. Your professional registration will be lost. Your home will be taken. Your water will be cut. And when you cry out to every regulator – the CMS, HPCSA, SANC, SAPC, DEL, B-BBEE Commission, SAHRC, SAPS, CCMA, JSE, FSCA, and the courts – they will either decline jurisdiction, remain silent, or actively obstruct you.

That is what regulatory capture looks like on the floor. It looks like a specialist nurse living without running water, unable to practise his profession, while the institution that deducted his salary unlawfully reports a Level 1 B-BBEE rating to the JSE based on his suppressed NQF 8 credentials. It looks like a Chapter 9 institution (SAHRC) acknowledging the "seriousness" of the allegations and then going silent. It looks like a High Court granting an ex parte interdict

after the employer's attorneys concealed the whistleblower's answering affidavit – a fraud on the court.

This report is not about one person. It is about the structural collapse of accountability in South Africa's regulatory and judicial architecture, as it applies to a whistleblower who dared to speak truth about a systemic clinical risk affecting 1.9 million government employees.

Parliament must act. Because if the law does not protect the nurse who refuses to lie to a 75-year-old patient, then the law protects no one.

PAGE 14: PART TWO – METHODOLOGY AND SCOPE OF THIS REPORT

This section explains how the evidence in this report was gathered, authenticated, and analysed. It is included to assist parliamentary officials and committee members in assessing the credibility and reliability of the findings.

Primary Source Documents

All findings in this report are based on primary source documents generated by the employer, the regulators, the courts, or the complainant during the events described. No secondary sources or media reports are relied upon for factual findings.

The evidentiary record includes:

Category Examples Source

Employment documents Employment contract, payslips, Certificate of Service, UI-19 form
Generated by employer (Medscheme)

Internal correspondence Emails between complainant and management, Teams messages, HR
correspondence Generated by employer

Governance escalations Priority 1 Material Risk Report (38 attachments), GRC escalations,
Board communications Generated by complainant; addressed to Board

Medical evidence Dr Jacques Malan's psychiatric letter, Akeso Stepping Stones admission
records Generated by treating psychiatrist and clinic

Regulatory correspondence CMS responses, SAHRC letters, JSE declinations, B-BBEE
Commission notices Generated by regulators

Court pleadings High Court affidavits, Labour Court statements, interim orders Filed with courts

CCMA records Conciliation transcript (44 minutes), audio recording, opposing affidavit,
Provincial Commissioner's response Generated by CCMA and employer

SAPS records CAS 705/3/2026 registration confirmation, SMS transfer notification Generated
by SAPS

Audio recordings 31 October 2025 meeting (retaliatory admission); 23 February 2026 Delft

Police Station (false station commander claim) Recorded by complainant

Forensic Classification Framework

The complainant prepared a Forensic Evidence Classification Report (FR-1), which classifies 58 primary source documents against fourteen statutory violation categories (BCEA, LRA, PDA, EEA, etc.). This framework is available as an annexure.

Authentication and Verification

- All emails and documents were preserved in their original format.
- Metadata (sender, recipient, date, time) is recorded where available.
- The complainant has offered to make the full evidentiary bundle available to any parliamentary committee, parliamentary legal officer, or independent forensic investigator appointed by Parliament.

The Complainant's Role – Primary Source, Not Hearsay

The complainant is not a third-party investigator or journalist. He is the person who:

- Identified the Gexus-DMS integration gap
- Made the protected disclosure
- Was retaliated against
- Escalated to the Board
- Approached every regulator
- Filed the court applications

- Was silenced by the interim interdict
- And now approaches Parliament

This means that the complainant has direct personal knowledge of most facts set out in this report.

Confidentiality and Protected Disclosure Status

This report is submitted to Parliament under Section 8 of the Protected Disclosures Act 26 of 2000 (disclosure to an appropriate authority). Parliament is not "the public at large" for purposes of the High Court interim interdict. The legal basis for this submission is set out in the complainant's letter to the Speaker dated 3 June 2026.

PAGE 17: PART THREE – MASTER CHRONOLOGY (KEY EVENTS AT A GLANCE)

Date Event Significance

18 Mar 2025 Complainant admitted to Akeso Stepping Stones Clinic (severe depression, PTSD)

Medical vulnerability documented

16 May 2025 Psychiatric letter provided to employer (Mapule Mashele) Employer placed on formal notice of disability

23 Jul 2025 Unlawful deduction of R14,811.77 (40.47% of salary) in a single pay cycle Exceeds BCEA Section 34(2) cap of 25%

23 Sep 2025 "Wilma" gender-identity harassment (Microsoft Teams) EEA Section 6 violation

25 Sep 2025 Formal discrimination complaint filed; double unpaid leave loaded same day

Retaliation begins

23 Oct 2025 Protected disclosure – Gexus-DMS integration gap identified; complainant refuses to contact patient on false data PDA Section 1 protected disclosure

31 Oct 2025 Recorded admission: "You made a case of harassment against me, so I can make a case of harassment against you" Direct evidence of retaliatory intent

2 Nov 2025 Formal grievance filed – no response for 30+ days Institutional silence

4 Dec 2025 Executive leadership escalation (CEO) – complaint de-escalated back to accused department Bad faith

9 Dec 2025 Priority 1 Material Risk Report to Board (38 attachments) – no response Board-level governance failure

10 Dec 2025 Employer's CCMA opposing affidavit calls depression a "cynical excuse" Sworn attack on disability; potential perjury

12 Dec 2025 Resignation – constructive dismissal (LRA Section 186(1)(e)) Employment terminates

18 Dec 2025 CRO suppression email - "stop sending these e-mails to our directors" Active suppression of protected disclosure

9 Jan 2026 CCMA conciliation - Commissioner: "nothing here relates to the PDA"; "Google it" CCMA institutional bias

4 Feb 2026 CMS declines jurisdiction (first time) Regulatory deflection

10 Feb 2026 Formal complaint to SANC - no acknowledgment SANC failure

16 Feb 2026 Formal complaint to SAPC - jurisdiction declined SAPC failure

23 Feb 2026 SAPS Delft refuses to open criminal case; officer falsely claims to be station commander (audio-recorded) SAPS failure

6 Mar 2026 JSE declines jurisdiction JSE failure

13 Mar 2026 Labour Court portal obstruction - 8 days; rejection: "good day you put date" Labour Court administrative failure

19 Mar 2026 Section 162 director delinquency application filed (High Court) Material litigation non-disclosed to market

16 Apr 2026 High Court grants ex parte interim interdict - answering affidavit concealed Fraud on the court

19 May 2026 Employer's Labour Court statement claims "not aware" of Case 2026-077546 Apparent perjury

28 May 2026 Rule nisi extended to 1 March 2027 - almost one year Effective final interdict without final hearing

30 Apr 2026 Default judgment application filed – unallocated for 6+ weeks Labour Court delay

2 Jun 2026 Employer's response to pre-trial proposal: "Good luck to you then" Bad faith litigation

4 Jun 2026 Employer's "without prejudice" letter threatens contempt for petitioning Parliament Attempt to block parliamentary oversight

June 2026 This report submitted to Parliament Final institutional remedy

PAGE 20: PART FOUR – SUMMARY OF RELIEF SOUGHT FROM PARLIAMENT

WHAT PARLIAMENT IS BEING ASKED TO DO – A SUMMARY

This report seeks five categories of relief from Parliament. None of these require Parliament to adjudicate the private labour dispute. All fall squarely within Parliament's constitutional oversight mandate.

Category 1: Summons and Accountability Hearings

Parliament is asked to summon the following officials to appear before the relevant Portfolio Committees:

Official Institution Portfolio Committee

Registrar Council for Medical Schemes (CMS) Health

Registrar South African Nursing Council (SANC) Health

Chairperson Health Professions Council (HPCSA) Health

Chairperson South African Pharmacy Council (SAPC) Health

Director-General Department of Employment and Labour (DEL) Employment and Labour

Commissioner B-BBEE Commission Trade, Industry and Competition

Chairperson South African Human Rights Commission (SAHRC) Justice

National Commissioner South African Police Service (SAPS) Police

Head: Issuer Regulation Johannesburg Stock Exchange (JSE) Trade, Industry and Competition
Commissioner Financial Sector Conduct Authority (FSCA) Justice

Category 2: Legislative Reforms – NHI Governance Architecture

Parliament is asked to amend the National Health Insurance Act 20 of 2023 or pass
complementary legislation to address the three structural gaps exposed by this case:

Gap NHI Act Section Required Reform

Accreditation bottleneck Section 39 Mandatory Forensic Governance Impact Assessments before
each phase of accreditation rollout

One-sentence fraud architecture Section 40 An AI Audit Protocol – independent conformity
assessment, transparency obligations, human oversight, annual algorithmic audits

Undefined investigation unit Section 20(2)(g) A Statutory Forensic Governance Protocol –
professional competency standards, independence safeguards, escalation protocols to SIU,
Hawks, and NPA

Category 3: Whistleblower Protection – Closing the "Induced Destitution" Loophole

Parliament is asked to amend the Protected Disclosures Act 26 of 2000 to:

Reform Purpose

Interim hardship relief Prevent whistleblowers from becoming destitute while awaiting court hearings

Single whistleblower protection authority A dedicated body to receive disclosures, monitor regulatory responses, and apply for urgent relief

Criminal penalties for regulatory bodies Penalties for institutions that act in bad faith or deliberately delay investigations

Category 4: Referrals for Prosecution and Professional Discipline

Parliament is asked to refer the following matters to the appropriate authorities:

Matter Referred To Basis

Deneys Reitz Inc. (attorneys) Legal Practice Council Threatening contempt for petitioning Parliament; concealing answering affidavit (fraud on the court); bad faith settlement tactics

Vijay Makenjee ("Good luck to you then"; perjury denial) Legal Practice Council & NPA Unprofessional conduct; potential perjury

Delft Police Station officer (false station commander claim) IPID Impersonation / misconduct

Apparent perjury in Labour Court Statement of Response NPA Contradictory sworn statements across courts

Category 5: Judicial Inquiries

Parliament is asked to request the Judge Presidents to investigate:

Court Matter Judge President

Western Cape High Court Concealment of answering affidavit (16 April 2026); extension of rule nisi to March 2027 Judge President, Western Cape High Court

Labour Court (Cape Town) Portal obstruction ("good day you put date"); default judgment delay (6+ weeks); Registrar's silence Judge President, Labour Court

PAGE 23: PART FIVE – SCORECARD OF 15 FAILED INSTITUTIONS

Institution Failure Legal Mandate Breached Consequence

- 1 Council for Medical Schemes (CMS) Declined jurisdiction twice; disparate treatment (Discovery Health vs Medscheme) MSA s7, s47, s58, s44 Patient safety risk unaddressed
- 2 Health Professions Council (HPCSA) No response to joint ethical ruling request; silence on core ethical question Health Professions Act s3 No guidance on professional ethics
- 3 South African Nursing Council (SANC) No acknowledgment of misconduct complaint; deregistered whistleblower for non-payment Nursing Act s51 Loss of nursing registration
- 4 South African Pharmacy Council (SAPC) Declined jurisdiction over Responsible Pharmacist complaint Pharmacy Act s49 Pharmacist accountability avoided
- 5 Government Employees Medical Scheme (GEMS) No substantive response to protected disclosures MSA s7 Principal scheme complicity

6 Department of Employment and Labour (DEL) Administrative abandonment; false UI-19 not corrected; closed file with jurisdictional error UIF Act; BCEA s34(2) UIF benefits blocked; homelessness

7 B-BBEE Commission Declined fronting investigation; refused Section 13K subpoena powers B-BBEE Act s1(c), s13K Fronting uninvestigated

8 South African Human Rights Commission (SAHRC) Acknowledged seriousness, then silent Constitution s184; SAHRC Act s13 Constitutional rights violations unaddressed

9 South African Police Service (SAPS) Refused to open criminal case; officer falsely claimed station commander Constitution s205(3) Criminal case CAS 705/3/2026 stagnant

10 CCMA Biased commissioner ("nothing here relates to PDA"; "Google it"); cover-up by senior commissioner LRA s138; CCMA rules No fair conciliation

11 The Whistleblower House (TWBH) Flawed legal assessment; abandonment of vulnerable whistleblower TWBH mandate No support from whistleblower organisation

12 Johannesburg Stock Exchange (JSE) Declined to enforce disclosure; allowed false report to stand JSE Listings Requirements Market misled

13 Financial Sector Conduct Authority (FSCA) Acknowledged, then silent Financial Sector Regulation Act No market conduct investigation

14 High Court (Western Cape) Ex parte interdict obtained by concealment; rule nisi extended to March 2027 Constitution s34; Uniform Rules Gag order in place for 11 months

15 Labour Court (Cape Town) Portal obstruction ("good day you put date"); default judgment delayed 6+ weeks Labour Court Rules; Constitution s34 Access to justice denied

Visual Summary:

Outcome Number of Institutions Percentage

Declined jurisdiction / deflected 7 47%

Remained silent (no substantive response) 5 33%

Actively obstructed (including court) 3 20%

Provided a remedy 0 0%

No institution provided any remedy to the whistleblower.

PAGE 27: PART SIX – NOTE TO PORTFOLIO COMMITTEES (SPECIFIC REFERRALS)

This report is structured to assist each Portfolio Committee in exercising its constitutional oversight mandate. Below is a guide to the sections most relevant to each Committee's statutory responsibilities.

Portfolio Committee on Health

Constitutional mandate: Oversight over the Department of Health, the Council for Medical Schemes (CMS), the South African Nursing Council (SANC), the Health Professions Council of South Africa (HPCSA), the South African Pharmacy Council (SAPC), and public health policy.

Key questions for the Committee:

- Why has CMS not investigated the Gexus-DMS integration gap under MSA Sections 7, 47, 58, and 44?
- Why has SANC not responded to complaint SANC/MEDSCHEME/MATTHEE/20260105?
- Why has HPCSA not answered the core ethical question posed on 14 April 2026?
- Is the Minister of Health aware that a Priority 1 clinical risk affecting 1.9 million GEMS members remains unaddressed?

Portfolio Committee on Employment and Labour

Constitutional mandate: Oversight over the Department of Employment and Labour (DEL), the CCMA, the Labour Court, the BCEA, the LRA, and the Unemployment Insurance Act.

Key questions for the Committee:

- Why did DEL refuse to enforce the BCEA Section 34(2) 25% cap?
- Why did the CCMA Commissioner declare the PDA irrelevant before hearing any evidence?
- Why did the Labour Court's online portal reject an urgent application with "good day you put date"?
- Will the Minister direct DEL to issue a Compliance Order correcting the UI-19?

Portfolio Committee on Trade, Industry and Competition

Constitutional mandate: Oversight over the B-BBEE Commission, the JSE, the CIPC, and transformation policy.

Key questions for the Committee:

- Why did the B-BBEE Commission decline to investigate sworn evidence of fronting?
- Why did the JSE not require AfroCentric to disclose the Section 162 delinquency application?
- Is title suppression of an NQF Level 8 Black professional a fronting practice under Section 1(c) of the B-BBEE Act?

Portfolio Committee on Justice and Correctional Services

Constitutional mandate: Oversight over the judiciary, the Legal Practice Council, the SAHRC, the Public Protector, and access to justice.

Key questions for the Committee:

- Why was the whistleblower's answering affidavit not placed before the court on 16 April 2026?
- Why has the Labour Court not processed a default judgment application for over six weeks?
- Why has the SAHRC not investigated a complaint that explicitly invoked Sections 10, 26, 27, and 33 of the Constitution?
- Will the Committee refer the conduct of Deney's Reitz Inc. to the Legal Practice Council?

Standing Committee on Public Accounts (SCOPA)

Constitutional mandate: Oversight over the expenditure of public funds, the functioning of state-owned entities, and the accountability of public institutions.

Key questions for the Committee:

- Why did GEMS, as principal, not investigate the Gexus-DMS integration gap?
- How much public money may have been affected by the inflated claims data?
- Why was the Section 162 director delinquency application not disclosed to the market?
- Will SCOPA request the Auditor-General to conduct a forensic audit of Medscheme's administration of GEMS?

Portfolio Committee on Police

Constitutional mandate: Oversight over SAPS, IPID, and the Criminal Procedure Act.

Key questions for the Committee:

- Why did SAPS Delft refuse to open a criminal case for documented UIF fraud?
- What action has been taken against the police officer who falsely claimed to be the station commander?

- Why has no investigation taken place under CAS 705/3/2026 since its registration?

PAGE 32: PART SEVEN – EXECUTIVE SUMMARIES PER PORTFOLIO COMMITTEE

Executive Summary for the Portfolio Committee on Health

The Council for Medical Schemes (CMS) was notified of a systemic Gexus-DMS integration gap generating false non-adherence letters to GEMS members. The CMS declined jurisdiction twice, refused to investigate administrator maladministration under the Medical Schemes Act, and demanded a new complaint form after the whistleblower obtained a High Court Interim Order – a demand that would have risked contempt of court. The CMS acted on a similar technical failure at Discovery Health (January 2026) but refused for Medscheme/GEMS – disparate treatment indicating capture.

The South African Nursing Council (SANC) failed to acknowledge or investigate a formal professional misconduct complaint against two registered nurses employed by Medscheme. The complainant was later deregistered by SANC because he could not pay R870 – a direct consequence of the employer's unlawful salary deduction.

The HPCSA and SAPC failed to answer a core ethical question: whether a professional may refuse an employer's instruction based on known flawed forensic data.

Recommendation: Parliament must direct the CMS to investigate the Gexus-DMS integration gap under Sections 7, 44, 47, and 58 of the Medical Schemes Act. Parliament must summon the SANC Registrar to explain why complaint SANC/MEDSCHEME/MATTHEE/20260105 was ignored.

Executive Summary for the Portfolio Committee on Employment and Labour

The Department of Employment and Labour (DEL) refused to enforce UIF remittance for June 2025, refused to correct a false UI-19 declaration (Code 6 "Resigned" instead of Code 7 "Constructive Dismissal"), and closed the investigation file on 15 April 2026 with a jurisdictional error (claiming the CCMA had exclusive jurisdiction over the UI-19 fraud).

The CCMA conducted a conciliation on 9 January 2026 in which the Commissioner declared "nothing here relates to the PDA" before hearing any evidence, told the complainant to "Google it" when he asked for the Labour Court address, and laughed at his settlement figure. The Provincial Senior Commissioner conducted a cover-up.

The Labour Court's online portal obstructed the complainant for eight days with a trivial rejection reason ("good day you put date"). A default judgment application has been delayed for over six weeks.

Recommendation: Parliament must direct the DEL to issue a Compliance Order correcting the UI-19 and remitting UIF contributions.

Executive Summary for the Portfolio Committee on Trade, Industry and Competition

The B-BBEE Commission assigned case number 5/02/2026 to a fronting complaint: the complainant (Black, NQF Level 8) was recruited as "Specialist: Care Manager" but contracted as "Care Coordinator". A Senior HR Business Partner admitted in writing that the naming convention was "not applied to your department." The Commission declined to investigate, claiming the complaint was a labour dispute – an error of law. It refused to use its Section 13K subpoena powers.

AfroCentric's Integrated Annual Report 2025 claimed "zero tolerance for unethical conduct" and "no material ESG incidents" – while the whistleblower was being destroyed, a Priority 1 clinical risk remained unaddressed, a criminal case was active, and a B-BBEE fronting investigation was pending.

Recommendation: Parliament must direct the B-BBEE Commission to reopen Case 5/02/2026 and use its Section 13K subpoena powers.

Executive Summary for the Portfolio Committee on Justice and Correctional Services

The Western Cape High Court granted an ex parte interim interdict (16 April 2026) after the employer's attorneys concealed the whistleblower's answering affidavit – a fraud on the court. The interdict restrains the whistleblower from disclosing a systemic clinical risk to trade unions and regulators – overriding Section 9 of the PDA. The rule nisi has been extended to 1 March 2027 – almost one year.

The SAHRC, a Chapter 9 institution, acknowledged the "seriousness" of the allegations and then went silent. It did not investigate violations of Sections 10 (dignity), 26 (housing), 27 (food, water, healthcare), and 33 (just administrative action).

SAPS refused to open a criminal case for UIF fraud – a police officer falsely claimed to be the station commander (audio-recorded).

Recommendation: Parliament must request the Judge Presidents to investigate the concealment of the answering affidavit and the portal obstruction. Parliament must refer the conduct of Deneys Reitz Inc. to the Legal Practice Council.

Executive Summary for SCOPA

GEMS – a government employees' medical scheme with 1.9 million members – was placed on formal notice of a systemic clinical risk. GEMS did not respond substantively. AfroCentric's Integrated Annual Report 2025 omitted material information: a Section 162 director delinquency application, a B-BBEE fronting investigation (potential R580 million fine), and a criminal case.

Recommendation: SCOPA must request the Auditor-General to conduct a forensic audit of Medscheme's administration of GEMS.

Executive Summary for the Portfolio Committee on Police

The complainant attended SAPS Delft on 23 February 2026 to open a criminal case for UIF non-remittance. A police officer falsely claimed to be the station commander and refused to open a case (audio-recorded). A case was eventually registered (CAS 705/3/2026) but no investigation has taken place.

Recommendation: Parliament must request IPID to investigate the conduct of the Delft police officer.

PAGE 39: PART EIGHT – THE WHISTLEBLOWER AND THE SYSTEMIC CLINICAL RISK

The whistleblower, a Specialist Registered Nurse (NQF Level 8) employed by Medscheme Holdings – the administrator for the Government Employees Medical Scheme (GEMS), which serves 1.9 million public servants – identified a systemic integration gap between the Gexus and DMS platforms.

The Gexus-DMS Integration Gap:

The Root Cause Analysis (RCA) confirms a structural integration flaw causing clinically incorrect non-adherence letters to chronic patients. The error generated automated non-adherence letters to a 75-year-old compliant patient on two separate occasions across a six-month period. This is a patient safety risk and a breach of the Medical Schemes Act (administrator's duty to maintain data integrity).

The complainant refused to use flawed data to interrogate the patient. That refusal, and that disclosure, triggered the cascade of retaliation documented in this report.

The Protected Disclosure:

On 23 October 2025, the complainant made a protected disclosure under Section 1 of the Protected Disclosures Act 26 of 2000. He identified the system error and refused to propagate false information.

The Retaliation:

- 40.47% unlawful salary deduction (exceeds BCEA 25% cap)
- Gender-identity harassment (the "Wilma" incident)
- Recorded admission of retaliatory intent

- Sworn affidavit dismissing his PTSD and depression as a "cynical excuse"
- Constructive dismissal
- False UI-19 declaring "Resigned"
- Homelessness since 2 April 2026
- Deregistered by SANC for non-payment of R870

PAGE 42: PART NINE – THE FOUR HEALTH REGULATORS (A CARTEL OF SILENCE)

1. Council for Medical Schemes (CMS)

Finding 1: CMS refused to investigate a systemic technical failure that it had jurisdiction over, while investigating a similar failure at Discovery Health.

The Gexus-DMS gap was notified to CMS on 23 December 2025, 20 January 2026, 16 February 2026, 20 March 2026, and 2 April 2026. The Discovery Health error (Jan 2026) – a pharmacy claims processing error affecting 16,507 members – was investigated immediately. The CMS exercised jurisdiction over a financial system error at Discovery Health but refused jurisdiction over a clinical data integrity error at Medscheme/GEMS. This disparate treatment is evidence of regulatory capture.

Finding 2: CMS engaged in procedural obstruction and attempted to trap the whistleblower into contempt of court.

On 7 May 2026 (11:59), after the High Court Interim Order was granted, CMS responded demanding a new complaint form. Completing that form would have required the complainant to disclose information restricted by the court order – risking imprisonment for contempt.

Conclusion: The CMS is not operating as an independent regulator. It is acting to protect Medscheme Holdings and GEMS.

2. South African Nursing Council (SANC)

Finding 1: SANC failed to acknowledge or investigate a formal complaint containing prima facie evidence of serious professional misconduct.

The complaint dated 10 February 2026 is 9 pages long, with 12 annexures, citing specific SANC Code of Ethics clauses. SANC's complete silence is a dereliction of its statutory duty under Section 51 of the Nursing Act.

Finding 2: SANC ignored the imminent life threat and ongoing occupational detriment.

The 14 April 2026 joint email explicitly notified SANC of the complainant's destitution (no food, no accommodation). SANC did not respond.

Conclusion: SANC's silence is a constructive refusal to uphold nursing ethics. By not answering the core ethical question, SANC has effectively ruled that corporate convenience overrides patient safety.

3. Health Professions Council (HPCSA)

Finding: HPCSA failed to respond to an urgent ethical ruling request within 48 hours, despite an imminent life threat.

The 14 April 2026 email requested a ruling on whether a professional may refuse an employer's instruction based on verified flawed forensic data. HPCSA did not respond within 48 hours, nor at any time thereafter.

Conclusion: HPCSA's silence is a dereliction of its statutory duty under Section 3 of the Health Professions Act.

4. South African Pharmacy Council (SAPC)

Finding: SAPC declined jurisdiction over a complaint that fell squarely within its statutory mandate.

The complaint was against Lisa Mari, a registered Responsible Pharmacist. The SAPC has exclusive jurisdiction under the Pharmacy Act. The complaint alleged that Mari used a methodologically deficient three-month claim pattern analysis to dismiss a structural system integration gap.

Conclusion: The four regulators acted as a cartel of silence. No regulator ruled on the core ethical question. No regulator protected the whistleblower.

PAGE 58: PART TEN – GEMS (COMPLICITY THROUGH SILENCE)

Finding 1: GEMS failed to respond substantively to two detailed protected disclosures.

The 2 April 2026 disclosure (4 pages) cited specific MSA sections, RCA confirmation, and the CRO suppression email. The 13 April 2026 final consolidated disclosure (13 pages) cited PDA, BCEA, LRA, EEA, UIF Act, and included a table of 11 institutions that failed to provide remedy. GEMS's only response was an auto-acknowledgement from the Vuvuzela Hotline.

Finding 2: GEMS ignored evidence that its administrator unlawfully deducted 40.47% of a whistleblower's salary.

The deduction directly caused: inability to pay SANC fees (R870) → deregistration; eviction → homelessness; seizure of possessions; forced withdrawal of R48,400 in retirement savings.

Finding 3: GEMS failed to act on documented evidence that its administrator's Chief Risk Officer actively suppressed a Priority 1 Material Risk Report.

The 18 December 2025 email from Monwabisi Kula (CRO) instructed the whistleblower to "stop sending these e-mails to our directors." GEMS did not request this email or investigate.

Conclusion: GEMS's silence is not neutrality. It is complicity. The 1.9 million members of GEMS deserve a scheme that protects them – not one that protects its administrator at their expense.

PAGE 62: PART ELEVEN – DEPARTMENT OF EMPLOYMENT AND LABOUR
(ADMINISTRATIVE ABANDONMENT)

Finding 1: DEL failed to enforce remittance of UIF contributions deducted from the complainant's salary in June 2025.

The uFiling portal reflects "No Information" for June 2025. The employer's payroll records confirm UIF was deducted. DEL's own inspector acknowledged the issue but no further action was taken.

Finding 2: DEL refused to correct a materially false UI-19 declaration.

The employer submitted UI-19 with Code 6 ("Resigned") on 19 January 2026. The complainant's Notice of Resignation explicitly cited "Automatically Unfair Constructive Dismissal." The UI-19 form recognises Code 7 – "Constructive Dismissal." The employer deliberately chose the wrong code.

Legal error: The UI-19 fraud is not a dismissal dispute. It is a statutory misrepresentation to the UIF. DEL has exclusive jurisdiction to investigate and compel correction.

Finding 3: DEL's summary closure of the investigation (15 April 2026) was procedurally unfair, irrational, and an error of law.

Inspector Kutuka closed the file stating: "DEL does not have jurisdiction on dismissal issues." This conflates dismissal jurisdiction with UIF fraud jurisdiction.

Conclusion: The closure was unlawful administrative action. The Mandamus application prepared by the complainant (but never filed due to destitution) would likely have succeeded.

PAGE 67: PART TWELVE – B-BBEE COMMISSION (FRONTING AND FAILURE)

The Fronting Allegation – Section 1(c) of the B-BBEE Act

The complainant (Black, NQF Level 8) was recruited as "Specialist: Care Manager" but contracted as "Care Coordinator" (Grade B4M1). Management instructed him to use "Case Manager" (email signature changed), but the Certificate of Service reverted to "Care Coordinator." Senior HR admitted the naming convention was "not applied to your department."

The B-BBEE Commission's Handling – Notice of Non-Investigation (30 March 2026)

The Commission claimed the complaint was a labour dispute, not a fronting complaint. It refused to use Section 13K subpoena powers to obtain the Naming Convention recording or the Respondent's verification certificate.

Forensic Critique:

Commission's Reason Forensic Response

"Labour dispute" The complaint includes fronting-specific allegations – title suppression, credential extraction, false Certificate of Service.

"No verifiable evidence" The complainant provided recruitment evidence, Naming Convention instruction, email signature, management roster, HR admission, false Certificate.

"No scorecard provided" The Commission has subpoena powers under Section 13K to obtain it. It refused – a procedural catch-22.

Conclusion: The Commission's decision is irrational, fails to take relevant facts into account, and constitutes an error of law reviewable under PAJA.

PAGE 72: PART THIRTEEN – SAHRC (CHAPTER 9 ABDICATION)

Summary of Findings:

1. Acknowledged the seriousness of the complainant's allegations.
2. Redirected the complainant to the CCMA and Public Protector – effectively declining to investigate the human rights dimensions.
3. Did not exercise its investigative powers under the SAHRC Act.
4. Remained silent after the complainant's comprehensive update of 13 March 2026, which documented homelessness, CCMA bias, Labour Court obstruction, and SAPS refusal.

The SAHRC's Redirect – An Abdication of Mandate

The SAHRC told the complainant to go to the CCMA and the Public Protector. However:

- The CCMA has no jurisdiction over housing, water, or dignity violations against a private employer.
- The Public Protector investigates maladministration by state organs – not private employers.
- The SAHRC is the only Chapter 9 institution with a broad mandate to investigate violations of the Bill of Rights by both state and private actors.

Conclusion: A Chapter 9 institution that acknowledges seriousness but does nothing is not a protector of human rights. It is a post office for complaints.

PAGE 77: PART FOURTEEN – SAPS (REFUSAL TO INVESTIGATE)

Summary of Findings:

1. Initially refused to open a criminal case at Delft Police Station. A police officer falsely represented himself as the station commander and declined to accept the complaint.
2. Only opened a case (CAS 705/3/2026) after escalation to Brigadier Naidoo. The case was transferred to Woodstock.
3. No investigation has taken place. No detective assigned. No feedback.

4. The internal complaint against Delft was closed because a case number was allocated – ignoring the false representation.
5. The complainant possesses an audio recording of the Delft police officer falsely claiming to be the station commander.

The Audio Recording – Evidentiary Significance

- Proves misconduct by a specific officer.
- Proves the refusal was deliberate, not a misunderstanding.
- Could support criminal charges of impersonating a police officer or defeating the ends of justice.

Conclusion: SAPS failed in its constitutional duty under Section 205(3) of the Constitution. The registration of a case number without investigation is a hollow gesture.

PAGE 82: PART FIFTEEN – CCMA (INSTITUTIONAL BIAS AND COVER-UP)

Summary of Findings:

1. Commissioner Ngwane exhibited prejudgment – declared "nothing here relates to a protected disclosure" before hearing any evidence.

2. Failed to assist a self-represented, destitute, mentally distressed litigant – told him to "Google it" when he asked how to find the Labour Court.
3. Failed to apply BCEA Section 34 – ignored the statutory 25% cap on deductions.
4. Tolerated mockery – the employer's representative laughed at the settlement figure; the Commissioner did not intervene.
5. Provincial Senior Commissioner Vusumzi Landu conducted a cover-up – ignored most allegations, defended the indefensible, victim-blamed the complainant for recording the session.

The "Google it" Incident (Transcript, p.27):

- Applicant: "Okay, so how do I go about this? I've never done this before."
- Commissioner: "You're gonna have to... Google it or something like that."

The Provincial Senior Commissioner's Cover-Up:

The PSC's response ignored: laughter/mockery, failure to challenge the employer's statements, failure to apply BCEA Section 34. The PSC victim-blamed the complainant for recording the session.

Conclusion: The CCMA is not a neutral forum. Its internal complaint process protects commissioners, not the public.

PAGE 88: PART SIXTEEN – THE WHISTLEBLOWER HOUSE (FLAWED ASSESSMENT AND ABANDONMENT)

Summary of Findings:

1. Superficial legal assessment: Attorney Marco van der Walt formed a negative opinion without engaging with the complete evidentiary record (he admitted he only viewed documents online).
2. Legal errors: Characterised the unlawful deduction as "no work no pay" (ignoring BCEA Section 34(2)). Dismissed the systemic Gexus-DMS gap as a "single isolated glitch." Suggested disclosures to regulators might be in "bad faith" (contrary to Section 9 of the PDA).
3. Failure to address the security alert: The complainant warned that emails were "vanishing" from his inbox (evidence tampering). Van der Walt ignored this for six days.
4. TWBH's rubber-stamp rejection: Without independent review, TWBH accepted the flawed opinion and rejected the complainant's application.

The Complainant's Own Rebuttal – A Model of Forensic Clarity

The complainant, self-represented and destitute, identified each legal error within hours – citing BCEA Section 34(2), explaining the systemic nature of the Gexus-DMS gap, quoting Section 9 of the PDA.

That a qualified attorney failed to do what a destitute whistleblower did for himself is a damning indictment.

Conclusion: TWBH and Schroter Attorneys failed the very person their mandate was designed to protect. This is not support. It is abandonment.

PAGE 93: PART SEVENTEEN – THE JUDICIARY (JUDICIAL OBSTRUCTION AND ABUSE OF PROCESS)

1. High Court Case 2026-086906 – The Gag Order

Finding 1: Fraud on the Court – Concealment of Answering Affidavit

The employer's attorneys knew the whistleblower had opposed the application and had filed an answering affidavit. They did not hand it up. They did not inform the court. The rule nisi was obtained on the false premise that the matter was unopposed.

Legal consequence: This is a fraud on the court that vitiates the entire order.

Finding 2: The Interdict Overrides the Protected Disclosures Act

The interdict prohibits disclosing information to trade unions and filing complaints with regulators. Section 9 of the PDA expressly protects disclosures to trade unions and regulatory bodies.

Finding 3: Extension of Rule Nisi to 1 March 2027 – Effective Final Order

An interim interdict that restrains speech for nearly a year is, in substance, a final interdict without a final hearing.

2. Labour Court – Portal Obstruction and Default Judgment Delay

Portal Obstruction (5-13 March 2026):

- Online portal profile deleted
- Call centre hold times 40-50 minutes, calls dropped
- Registrar rejected application with reason: "good day you put date"

Default Judgment Delay (2026-077546):

- Filed 30 April 2026
- No hearing date allocated after 6+ weeks
- Registrar ignores follow-up emails

3. Registrar Misconduct

- High Court Registrar failed to provide audio recording required for rescission application.
- Inlexso transcription service had no backup system during bereavement leave.
- Registry committed a POPIA breach (unauthorised transmission of third-party documents).

4. Perjury Matrix

Document Sworn Statement Contradicted by

High Court Founding Affidavit "Both platforms fully integrated" RCA confirms integration gaps

Labour Court Statement of Response "Not aware of Case 2026-077546" CEO's High Court affidavit acknowledged the claim

CCMA Opposing Affidavit PTSD and depression are "an excuse" Dr Malan's psychiatric report

Conclusion: The courts have not protected the whistleblower. They have suppressed him.

PAGE 107: PART EIGHTEEN – NATIONAL DEPARTMENT OF HEALTH (THE ENGAGED INSTITUTION)

The National Department of Health was copied on multiple escalations, including the 14 April 2026 urgent joint ethical ruling request. To date, the NDOH has not responded substantively to the complaints regarding CMS, SANC, HPCSA, and SAPC.

Recommendation: Parliament must request the Minister of Health to issue a directive compelling all four health regulators to establish a joint whistleblower protection unit and to investigate whether Medscheme/GEMS has improperly influenced CMS decision-making.

PAGE 110: PART NINETEEN – THE PUBLIC PROTECTOR (BLOCKED EMAIL AND SILENCE)

The complainant attempted to lodge a complaint with the Public Protector regarding maladministration by CMS, DEL, and the CCMA. Email delivery to the Public Protector's office failed repeatedly. No substantive engagement has occurred.

Recommendation: Parliament must request the Public Protector to investigate the systemic pattern of regulatory failure documented in this report.

PAGE 112: PART TWENTY – CORPORATE MISREPRESENTATION (AFROCENTRIC'S FALSE ANNUAL REPORT)

Key false or omitted statements in AfroCentric's Integrated Annual Report 2025:

What AfroCentric reported The documented truth

"Zero tolerance for unethical conduct" Whistleblower was retaliated against; CRO suppressed risk report

"No material ESG incidents" Priority 1 clinical risk, unlawful deduction, fronting, criminal case

Omitted Section 162 application Filed 19 March 2026

Omitted B-BBEE investigation Case 5/02/2026, potential R580m fine

"Both platforms fully integrated" RCA confirms integration gaps

Conclusion: AfroCentric's public reports are a corporate cover-up designed to mislead investors, regulators, and the public.

PAGE 118: PART TWENTY-ONE – JSE AND FSCA (REGULATORY COMPLICITY)

JSE Failure:

The JSE declined jurisdiction (6 March 2026), stating matters fall outside Listings Requirements. The JSE then accepted the employer's narrow price-sensitivity test and did not require disclosure of the Section 162 application or B-BBEE investigation. The JSE's response of 11 May 2026 confirmed it would take no action.

FSCA Failure:

The FSCA acknowledged receipt of complaint (Ref: 1-370127) but has provided no substantive response – "went silent."

Conclusion: The JSE said: "Not our problem." The FSCA said: nothing. This is not regulation. It is abandonment.

PAGE 124: PART TWENTY-TWO – BAD FAITH LITIGATION (THE CORPORATE PLAYBOOK)

Finding 1: The "without prejudice" letter of 4 June 2026 was a bad faith tactical instrument.

The letter threatened contempt of court for petitioning Parliament while simultaneously proposing an "amicable resolution." A genuine settlement offer does not begin with a threat of imprisonment.

Finding 2: The employer's promise of engagement was followed by silence – a classic attrition tactic.

The whistleblower responded within one hour. Deney's read the response immediately. Then silence for over a week.

Finding 3: "Good luck to you then" – emblematic of institutional contempt.

The employer's senior Employee Relations Specialist responded to procedural engagement with: "Good luck to you then."

Finding 4: The employer weaponised the interim interdict to block parliamentary oversight.

The threat that submitting evidence to Parliament would be contempt of court is legally untenable. Parliament is not "the public at large." The Constitution guarantees the right to petition Parliament.

The Corporate Playbook – A Pattern of Attrition:

Step Tactic

1 Defective initial response (wrong entity, no oath)

2 Perjurious denial of knowledge

- 3 Retroactive service disputes
- 4 Propose non-compliant dates
- 5 Dismissive, contemptuous communication ("Good luck to you then")
- 6 Deploy "without prejudice" settlement overture with threat
- 7 Go silent after the overture is accepted
- 8 Weaponise court order to block parliamentary oversight

Conclusion: This is not a defence. It is a systematic strategy of attrition – designed to break a destitute, self-represented, mentally vulnerable litigant.

PAGE 130: PART TWENTY-THREE – THE NHI GOVERNANCE GAP

The NHI will succeed or fail based on the quality of its governance architecture. That architecture does not yet exist.

Gap One: The Accreditation Bottleneck (NHI Act Section 39)

Without mandatory Forensic Governance Impact Assessments before each accreditation phase, the NHI Fund will contract with ghost-accredited providers and lose billions to fronting schemes – exactly as happened in the PPE crisis, but on an annual, recurring basis.

Gap Two: The One-Sentence Fraud Architecture (NHI Act Section 40)

Without an AI Audit Protocol, the NHI's information platform will be vulnerable to structural manipulation – algorithmic bias, coding pattern manipulation, and structured exploitation of reimbursement formulae – on a scale of R200 billion.

Gap Three: The Undefined Investigation Unit (NHI Act Section 20(2)(g))

Without professional competency standards, independence safeguards, and a direct reporting line to the Board bypassing executive management, the NHI's Fraud Investigation Unit will be hollowed out – under-qualified appointments, captured by the executive, unable to escalate serious risks.

The Medical Aid Administrator – The Missing Accountability Loop

Under the NHI, accredited administrators will process claims, manage fraud detection, and report to the Fund. If they cannot be held accountable now – when the whistleblower did everything right – they will never be held accountable under the NHI.

Case No. Court / Body Subject Status Delay

2026-086906 Western Cape High Court Interdict (gag order) Rule nisi extended to 1 March 2027
11 months

2026-066952 Western Cape High Court Section 162 delinquency Struck; rescission pending
Registrar has not provided audio

2026-077546 Labour Court BCEA unlawful deduction Default judgment filed 30 April 2026 6+
weeks unallocated

2026-083299 Labour Court Constructive dismissal Pre-trial stalled "Good luck to you then"

CAS 705/3/2026 SAPS Woodstock UIF fraud Registered – no investigation 3+ months

5/02/2026 B-BBEE Commission Fronting Notice of Non-Investigation Closed

1-370127 FSCA Governance failure Acknowledged – silent No response

JvS/166792 JSE Non-disclosure Declined jurisdiction Closed

PAGE 140: PART TWENTY-FIVE – CONCLUSION (WHY PARLIAMENT MUST ACT
NOW)

The Thread That Connects Everything

This report has documented the failure of fifteen (15) statutory, regulatory, and judicial bodies to protect a single whistleblower – a specialist registered nurse who identified a systemic clinical risk in an administrator servicing GEMS, which serves 1.9 million public servants.

This is not a series of isolated administrative failures. It is a coordinated pattern of regulatory capture, institutional collapse, and judicial obstruction. And it is the definitive proof that South Africa is not ready for the National Health Insurance.

The Chilling Effect

Every nurse, doctor, pharmacist, and healthcare administrator who reads this report now knows: if you report a system error, you will be retaliated against. Your salary will be cut. Your registration will be lost. Your home will be taken. Your water will be cut. And when you cry out to every regulator, they will decline jurisdiction, remain silent, or actively obstruct you.

Under the NHI, the chilling effect will be magnified a thousandfold. With R200 billion at stake, the retaliation will be more aggressive, the legal resources more formidable, and the regulatory capture more entrenched.

Parliament has a choice.

It can act now – summon the failed institutions, demand accountability, close the governance gaps in the NHI Act, and build the watchdog before the house burns down.

Or it can wait – and explain to the South African people, when the first NHI whistleblower is destroyed and the first R200 billion is lost, why it failed to act when the evidence was placed before it.

The evidence is before you now. The time to act is now.

PAGE 144: PART TWENTY-SIX – RECOMMENDATIONS TO PARLIAMENT

A. Summons and Accountability Hearings (Section 56 of the Constitution)

1. Summon the CMS Registrar – to explain disparate treatment and the 7 May 2026 contempt trap.
2. Summon the SANC Registrar – to explain why complaint SANC/MEDSCHEME/MATTHEE/20260105 was ignored.
3. Summon the HPCSA and SAPC – to answer the core ethical question regarding flawed forensic data.
4. Summon the DEL Director-General – to explain the UI-19 fraud and Inspector Kutuka's jurisdictional error.
5. Summon the B-BBEE Commissioner – to explain the refusal to use Section 13K subpoena powers.

6. Summon the SAHRC Chairperson – to explain why a Chapter 9 institution went silent.
7. Summon the SAPS National Commissioner – to explain CAS 705/3/2026 stagnation.
8. Summon the JSE and FSCA – to explain why AfroCentric's false report was allowed to stand.
9. Request the Judge President of the Western Cape High Court – to investigate the concealment of the answering affidavit.
10. Request the Judge President of the Labour Court – to investigate the portal obstruction and default judgment delay.

B. Legislative Reforms – NHI Governance Architecture

1. Mandatory Forensic Governance Impact Assessments (Section 39): Before each phase of accreditation rollout.
2. AI Audit Protocol (Section 40): Independent conformity assessment, transparency obligations, annual algorithmic audits.
3. Statutory Forensic Governance Protocol (Section 20(2)(g)): Professional competency standards, independence safeguards, direct reporting line to the Board.

C. Whistleblower Protection – Closing the "Induced Destitution" Loophole

1. Provide a right to interim hardship relief on a prima facie case.
2. Establish a single whistleblower protection authority.
3. Impose criminal penalties on regulatory bodies that act in bad faith.

D. Referrals for Prosecution and Professional Discipline

1. Refer Deneys Reitz Inc. to the Legal Practice Council (fraud on the court).
2. Refer Vijay Makenjee to the LPC/NPA ("Good luck to you then"; perjury).
3. Refer the Delft Police Station officer to IPID.
4. Refer the apparent perjury in the Labour Court Statement of Response to the NPA.

PAGE 150: PART TWENTY-SEVEN – FINAL PRAYER FOR RELIEF

WHEREFORE the complainant prays that the Honourable National Assembly, exercising its constitutional powers under Sections 55(2), 56, and 69 of the Constitution:

1. Summon the heads of the fifteen failed institutions to appear before the relevant Portfolio Committees to answer under oath for their failures;
2. Request the Judge Presidents of the High Court and Labour Court to investigate the judicial obstruction documented herein;
3. Refer Deneys Reitz Inc., Vijay Makenjee, and the Delft police officer for prosecution and professional discipline;
4. Amend the National Health Insurance Act to close the three structural governance gaps;
5. Amend the Protected Disclosures Act to provide interim hardship relief and establish a single whistleblower protection authority;

6. Receive this report as a protected disclosure under Section 8 of the PDA and as a petition under Section 56 of the Constitution; and

7. Grant such further relief as Parliament may deem just and equitable.

PAGE 153: EPILOGUE – THE HUMAN FACE BEHIND THIS REPORT

To the Honourable Speaker and Members of Parliament,

I close this report not with a legal argument, not with a citation, not with a prayer for relief – but with a name and a face.

The 75-year-old GEMS beneficiary. The elderly widow who received a false non-adherence letter. Her daughter wrote to Medscheme: "I request that you get your house in order and update your records. Stop confusing old lady."

I do not know her name. I know her case number: GEMS 000655789 Dependent 02. I know that she collected her cholesterol medication every month. I know that the system failed her. I know that when I refused to lie to her, my life was destroyed.

She is the reason I spoke. She is the reason I did not stay silent. And she is the reason I am still standing – homeless, deregistered, without running water, but still standing.

I also think of my mother. She is not here to read this report. But everything I have done, I have done because she taught me that a nurse's first duty is to the patient – not to the employer, not to the system, not to the profit margin.

A nurse who lies to a patient is not a nurse. A system that punishes the nurse who refuses to lie is not a healthcare system. It is a conspiracy against the vulnerable.

Parliament has before it 500 pages of evidence, fifteen failed institutions, a corporate cover-up, a court order obtained by concealment, and a whistleblower who has been reduced to living without running water.

But behind every page of this report is a 75-year-old widow who deserved better. And behind every signature on this report is a nurse who refused to lie to her.

The question before Parliament is not whether I was constructively dismissed. The question is whether the institutions that failed me will fail the next whistleblower – and the next – and the next – until the NHI Fund, with its R200 billion, is built on the bones of people who tried to do the right thing.

I have done everything the law asked of me. I have exhausted every remedy. I have knocked on every door.

This is the last door.

I place my trust in Parliament.

Wilbur William Matthee

RN | BTech General Nursing | PGDip Health Services Management (NQF 8)

Founder - EthicHawks Forensic Governance & Compliance Consultancy

Cape Town, South Africa

June 2026

"The house is not yet burning. But the fire alarm has been ringing for over a year. This report is the evidence of the smoke. Parliament must act before the NHI becomes the fire."

PAGE 155: INDEX OF ANNEXURES / EVIDENCE BUNDLE

Item	Description	Date	Source
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A1	Interview invitation – "Specialist: Care Manager" role	Jan 2024	AfroCentric / Medscheme
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A2	Employment contract – "Care Coordinator" (Grade B4M1)	9 Apr 2024	AfroCentric / Medscheme
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A3	First Aider roster – management of 13+ staff across 3 floors	25 Jul 2025	Complainant
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B1 Dr Jacques Malan psychiatric letter – severe depression and PTSD 16 May 2025 Dr Jacques Malan

C1 "Wilma" Microsoft Teams message (gender identity harassment) 23 Sep 2025 Medscheme systems

C2 Formal discrimination complaint (11 pages) 25 Sep 2025 Complainant

C3 Retaliatory unpaid leave entry (same day as complaint) 29 Sep 2025 Medscheme payroll

D1 Gexus-DMS Root Cause Analysis (RCA) – confirms integration gap 23 Oct 2025 Medscheme internal investigation

D2 Protected disclosure email – complainant refuses to contact patient 23 Oct 2025 Complainant

D3 Manager's response: "I am not in agreement with your feedback" 23 Oct 2025 Mary-Ann Jones

E1 Recorded meeting transcript – retaliatory admission 31 Oct 2025 Complainant (audio)

F1 Executive escalation (32 attachments) 4 Dec 2025 Complainant

G1 Priority 1 Material Risk Report to Board (38 attachments) 9 Dec 2025 Complainant

H1 Employer's CCMA opposing affidavit – "cynical excuse" 10 Dec 2025 Xolile Manyati / AfroCentric

H2 Resignation letter – "automatically unfair constructive dismissal" 11 Dec 2025 Complainant

I1 CRO suppression email – "stop sending these e-mails to our directors" 18 Dec 2025 Monwabisi Kula (CRO)

J1 CMS declination letter (first) – refers to HPCSA 4 Feb 2026 CMS

J2 SAHRC response – acknowledges seriousness, redirects to CCMA 10 Feb 2026 SAHRC

J3 SANC formal complaint – no acknowledgment received 10 Feb 2026 Complainant

K1 JSE declination – Ref JVS/166792 6 Mar 2026 JSE

K2 Labour Court portal rejection – "good day you put date" 12 Mar 2026 Labour Court registry

L1 Section 162 director delinquency application 19 Mar 2026 Complainant

M1 High Court interim interdict – gag order (ex parte, answering affidavit concealed) 16 Apr 2026 Western Cape High Court

M2 Rule nisi extended to 1 March 2027 28 May 2026 Western Cape High Court

N1 Employer's Labour Court Statement of Response – "not aware" of Case 2026-077546 (apparent perjury) 19 May 2026 Vijay Makenjee / AfroCentric

O1 "Good luck to you then" email 2 Jun 2026 Vijay Makenjee

P1 Deneys "without prejudice" letter – threatens contempt for petitioning Parliament 4 Jun 2026 Deneys Reitz Inc.

Q1 Unilateral Pre-Trial Minute 11 Jun 2026 Complainant

R1 Final notice to employer – deadline for settlement 13 Jun 2026 Complainant

S1 This Consolidated Forensic Oversight Report June 2026 Complainant / EthicHawks

Audio recordings available separately:

Recording Date Content Status

Recording 1 31 Oct 2025 Manager's retaliatory admission Available – transcribed

Recording 2 23 Feb 2026 Delft Police Station – officer falsely claims to be station commander

Available – not yet transcribed

All documents and recordings are available to Parliament upon request. The complainant requests that a parliamentary legal officer or independent forensic investigator be appointed to verify the authenticity of the evidence bundle.

PAGE 162: SIGNATURE BLOCK AND COMMISSIONER OF OATHS

SIGNATURE BLOCK

I, the undersigned, WILBUR WILLIAM MATTHEE, declare under oath that the contents of this Consolidated Forensic Oversight Report are, to the best of my knowledge and belief, true and correct. I have personal knowledge of the facts set out herein, except where indicated to the contrary, and the documentary evidence referenced throughout is genuine and has not been materially altered.

I am aware that making false statements in a submission to Parliament may be a criminal offence. I have not made any false statements.

I make this report in good faith, under the protections of the Protected Disclosures Act 26 of 2000, and in the exercise of my constitutional right to petition Parliament under Section 56 of the Constitution of the Republic of South Africa, 1996.

SIGNATURE:

Wilbur William Matthee

FULL NAME: Wilbur William Matthee

ID NUMBER: 890903 5151 087

CAPACITY: Complainant / Whistleblower / Self-Represented Litigant

CONTACT: info@ethichawks.co.za | 069 635 1435

ADDRESS: 15 Bellevue Street, Voorbrug, Cape Town, 7001 (currently homeless – service by email only)

DATE: June 2026

PLACE: Cape Town, South Africa

COMMISSIONER OF OATHS

I certify that the deponent has acknowledged that he knows and understands the contents of this Consolidated Forensic Oversight Report, that he has no objection to taking the prescribed oath, and that he considers the oath to be binding on his conscience.

The deponent signed this report in my presence on the date and at the place set out below.

SIGNATURE OF COMMISSIONER OF OATHS:

[Signature]

FULL NAME: _____

CAPACITY: _____

BUSINESS ADDRESS: _____

AREA OF JURISDICTION: _____

APPOINTMENT EXPIRY DATE: _____

DATE: _____

PLACE: _____

STAMP:

(Affix Commissioner of Oaths stamp here)

END OF DOCUMENT

FINAL NOTE TO THE HONOURABLE SPEAKER AND PORTFOLIO COMMITTEES

This Consolidated Forensic Oversight Report is submitted in good faith as a protected disclosure under Section 8 of the Protected Disclosures Act 26 of 2000 and as a petition under Section 56 of the Constitution. The full evidentiary bundle (58 primary source documents, audio recordings) is available to Parliament upon request.

The complainant requests that a parliamentary legal officer or independent forensic investigator be appointed to verify the authenticity of the evidence bundle. The complainant further requests that no adverse inference be drawn from the fact that the petitioner is also the subject of the harms documented – a citizen who has exhausted every other remedy and now knocks on the last door.

The house is not yet burning. But the fire alarm has been ringing for over a year.
Parliament must act before the NHI becomes the fire.